

**BUSINESS ASSETS**

VYTALIX — Investor pitch deck (14 slides)

Vytalix company-build documentation · reference
INVESTOR_PITCH_DECK

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

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Design per `/docs/03_BRAND_SYSTEM.md`. Every slide carries only claims traceable to `/FACTS_BASE.md`.
Speaker notes are for the founder.

Slide 1 — Title

Headline: VYTALIX · Technology for Life

Body: UK health-technology group · Pre-seed · [Month Year] · [Founder name, role]

Visual: Wordmark on deep-navy field; thin contour-line motif.

Notes: Thirty seconds. Who I am, what Vytalix is in one sentence, what I want from the meeting: a fit test, not a decision.

Slide 2 — The problem

Headline: Maternity care fails most often between appointments, not during them

Body: UK reviews (Ockenden, Kirkup) repeatedly cite communication and coordination failures · MBRRACE-UK shows persistent inequalities for Black and Asian women · Nigeria carries one of the world's highest maternal mortality burdens, with weak continuity between women and facilities

Visual: Three source cards with citations; no stock photography.

Notes: Cite sources by name. Do not quote a statistic you cannot footnote.

Slide 3 — Who we are

Headline: A group, not an app: services that fund the company and a product that defines it

Body: Advisory · Consulting · Health Solutions (MaternaLink, in development) · E-Learning (year 2)

Visual: Four-pillar diagram with the flow "services cash → product build".

Notes: Explain why services first: revenue, references, credibility, and it keeps the product honest.

Slide 4 — MaternaLink (in development)

Headline: A coordination layer between women, midwives and facilities

Body: Continuity of contact · two-way messaging with human escalation · clinician-governed education · programme dashboards · offline-tolerant, low-bandwidth, multilingual roadmap · non-diagnostic by design

Visual: Simple journey line: antenatal → birth → postnatal with touchpoints.

Notes: Say "in development" out loud. Say what it is not: not a diagnostic tool, not a medical device, not an AI predictor.

Slide 5 — Why the regulatory posture matters

Headline: Built to stay outside device classification, with a path if we ever cross the line

Body: Intended-use statement · clinical safety officer and hazard log (DCB0129) · UK GDPR, DPIA, data minimisation · DTAC readiness · AI risk-scoring kept as a research track behind ethics and a SaMD pathway

Visual: A line with "v1 coordination" on one side and "v3 research track" on the other.

Notes: This is the slide that answers the diligence question before it is asked.

Slide 6 — Market

Headline: Two entry markets with different buyers

Body: UK: NHS trusts and ICBs, private maternity, employers (services and pilots first) · Nigeria: state programmes, NGOs, insurers, telecom distribution (design to complement existing programmes) · Sizing in `/docs/05_MARKET_STRATEGY.md`, all ESTIMATE

Visual: Two columns; scored market table excerpt.

Notes: Name the Ekiti incumbent yourself. Investors respect it.

Slide 7 — Business model

Headline: Services now, platform licences later

Body: Advisory £1,200–£1,800/day · Consulting sprints £15k–£60k · Pilots £40k–£150k · Per-woman-per-year licences (UK £8–£40; Africa £3–£12 at scale) · E-learning B2B from year 2 (all ESTIMATE)

Visual: Revenue-mix bar for the Base scenario by year.

Notes: Be explicit that the £300/woman figure from a prior proposal is retired.

Slide 8 — Traction, honestly

Headline: What we have · what we are building · what is next

Body: Have: full company build, prototype under audit, prior ministry conversation in Nigeria · Building: entity, IP deed, advisory board, research report, first engagements · Next: first paid engagement (Day 45), one UK evaluation site and one Nigeria NGO cohort in design (Day 90)

Visual: Three-column card layout with dates.

Notes: No inflation. If a milestone has slipped, say so and say why.

Slide 9 — Competitive landscape

Headline: Record systems, consumer apps and donor programmes leave the coordination layer open

Body: Clinical records (BadgerNet, K2, Euroking) · consumer/pregnancy apps · donor-funded programmes (mDoc in Nigeria, MomConnect in South Africa) · consultancies for the services pillar · Vytalix's buildable advantages: evidence-first posture, interoperability, clinician governance, UK-plus-Africa design

Visual: 2×2: clinical depth vs reach across settings.

Notes: Say which advantages we cannot claim today.

Slide 10 — Technology

Headline: Lean, standard, secure

Body: Web/PWA front end · API and PostgreSQL · UK-region hosting with Nigeria residency options · messaging gateways (SMS, WhatsApp) · FHIR-aligned integration roadmap · Cyber Essentials and DSPT controls · no patient data to third-party AI without a DPIA

Visual: Architecture diagram from `/docs/07_TECHNOLOGY_ARCHITECTURE.md`.

Notes: Emphasise "what we will not build yet".

Slide 11 — Roadmap

Headline: Eight quarters, gated by evidence

Body: Q4 2026 foundations and services · 2027 MVP, service evaluation, NGO cohort · 2028 formal evaluation, seed, second market · 2029 scale decisions

Visual: Timeline with gates (entity, IP, pilot LOI, evaluation, funding).

Notes: Each gate names the claim it unlocks.

Slide 12 — Team and governance

Headline: Founder-led, with the advisory board being recruited now

Body: Founder [name, background] · Advisory board targets: obstetric/midwifery lead, NHS digital leader, Nigerian health-system leader, regulatory/QA adviser, investor · First hires funded by the round: product lead, fractional clinical safety officer

Visual: Org chart with "recruiting" markers; no invented people.

Notes: Never show a name without permission.

Slide 13 — Financials

Headline: Three scenarios, one model

Body: Base three-year revenue £1.47m; services 59% · Peak cash need £229k (Conservative) to £1.06m (Base) · Break-even not reached in 36 months in Base; sustained EBITDA depends on retainers and pilots converting · Full model: `/finance/Vytalix_Financial_Model_3yr.xlsx`

Visual: Annual revenue and EBITDA bars for three scenarios.

Notes: Lead with the assumptions, not the outputs.

Slide 14 — The raise and the ask

Headline: Pre-seed £250k–£500k to build the MVP and run two pilots

Body: Use of funds: product lead, clinical-safety artefacts, MVP build, two pilots, compliance · SEIS/EIS advance assurance applied for after incorporation (TO VALIDATE) · Milestones before close: entity, IP deed, first revenue, pilot LOI · Later rounds only on evidence

Visual: Use-of-funds donut; milestone list.

Notes: Ask directly: "Is this within mandate? What would you need to see to lead or co-invest?"

Appendix (on request): risk register, regulatory memo, pilot design, competitive detail, cap table illustration
(`/docs/09_FUNDING_STRATEGY.md`).